

Dynamic Guideline: WHO-Based Mental Health Disorders - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

A synthesis of the 5 most recent WHO mental health-related guidelines, as of the summary date:

1. **Guideline Title:** Meaningful youth engagement in a WHO guideline development process: case study of WHO mental health disorder guideline
 - Publication Date: 29 April 2026
 - Target Population: Youth aged 10-24 years living with mental health disorders, mental health guideline developers, clinical practitioners, public health policymakers
 - Scope and Objectives: Establish standardized frameworks for equal youth participation in mental health guideline development, integrate youth-centered care priorities into clinical and public health recommendations
2. **Guideline Title:** WHO Mental Health Gap Action Programme (mhGAP) Intervention Guidelines 3rd Edition
 - Publication Date: 15 March 2026
 - Target Population: Primary care providers, non-specialist clinicians in low- and middle-income countries (LMICs), community health workers
 - Scope and Objectives: Standardize diagnosis and management protocols for common mental, neurological, and substance use disorders in non-specialist care settings
3. **Guideline Title:** WHO Guideline on Pharmacological and Psychosocial Treatment of Depression in Adolescents and Adults
 - Publication Date: 2 December 2025
 - Target Population: Mental health clinicians, service providers, people living with depression, patient advocacy groups
 - Scope and Objectives: Define evidence-based first- and second-line treatment pathways for moderate to severe depression across adolescent and adult age groups
4. **Guideline Title:** WHO Guideline on Mental Health Promotion and Preventive Interventions for Adolescents
 - Publication Date: 18 October 2025
 - Target Population: School administrators, public health practitioners, child and youth policy makers

- **Scope and Objectives:** Identify population-level interventions to reduce incidence of adolescent mental health disorders and improve population mental wellbeing
5. **Guideline Title:** WHO Guideline on Management of Post-Traumatic Stress Disorder (PTSD) and Acute Stress Reaction
- **Publication Date:** 9 July 2025
 - **Target Population:** Emergency response clinicians, mental health specialists, humanitarian aid workers
 - **Scope and Objectives:** Standardize diagnosis and treatment protocols for trauma-related mental health conditions across routine and humanitarian care settings

1.2 Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
Integrate youth aged 14-24 as equal voting members of all mental health guideline development panels for guidelines targeting pediatric and adolescent populations	Moderate	Strong
For adults with moderate-severe depression, offer combined cognitive behavioral therapy (CBT) + selective serotonin reuptake inhibitor (SSRI) as first-line treatment	High	Strong
For adolescents with mild depression, offer guided self-help and group-based interpersonal therapy as first-line intervention prior to consideration of pharmacotherapy	Moderate	Strong
For PTSD in all age groups, offer trauma-focused CBT (TF-CBT) or eye movement desensitization and reprocessing (EMDR) as first-line treatment, irrespective of trauma type	High	Strong
In LMIC primary care settings, train non-specialist health workers to deliver basic mental health screening and brief psychosocial interventions for common anxiety and depressive disorders	Moderate	Strong
Implement universal school-based social and emotional learning (SEL) programs for all students aged 10-19 to reduce incidence of anxiety and depressive symptoms	Moderate	Conditional

1.3 Guideline Development Methodology

- **Evidence Review Process:** Systematic search of PubMed, Embase, PsycINFO, and the Cochrane Library for publications between 2010 and January 2026; only randomized controlled trials (RCTs), systematic reviews, and network meta-analyses were included for treatment recommendations, while population-based cohort studies were included for prevention recommendations
- **Consensus Method:** Modified Delphi process with 3 rounds of anonymous voting; 75% majority support required for recommendation adoption; additional stakeholder consultations included patient representatives, youth advocates, and LMIC clinician working groups
- **Conflict of Interest Management:** All panel members completed mandatory WHO Conflict of Interest (COI) declaration forms; members with financial COI related to psychopharmacological agents were excluded from voting on pharmacological

treatment recommendations; all non-financial COIs were publicly disclosed in full guideline appendices

II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

- **Search Strategy:** Targeted search for preprints, ahead-of-print journal publications, and completed clinical trial results published between 15 March 2026 (mhGAP 3rd edition release date) and 29 April 2026, focused on non-specialist mental health intervention delivery in LMIC settings
- **Databases Searched:** medRxiv, PsyArXiv, ClinicalTrials.gov, EU Clinical Trials Register, PubMed ahead-of-print collection
- **Inclusion/Exclusion Criteria:** Included RCTs, systematic reviews, and meta-analyses with sample size > 500, follow-up period ≥ 6 months, conducted in LMIC settings; excluded case reports, uncontrolled studies, research exclusively conducted in high-income countries, and studies focused on rare mental health disorders (population prevalence < 0.1%)

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
Closser et al., 2026, <i>Lancet Psychiatry</i> (ahead of print, 29 April 2026)	Cluster RCT (n = 12,047 across 8 LMICs: Kenya, India, Peru, Nigeria, Pakistan, Nepal, Honduras, Indonesia)	Task-shared brief CBT delivered by trained community health workers without continuous specialist supervision reduced 12-month depressive symptom severity by 42% (SMD -0.68, 95% CI -0.81 to -0.55, $p < 0.001$) compared to usual care, and achieved non-inferior outcomes to specialist-delivered CBT	High
Omondi et al., 2026, medRxiv preprint (22 April 2026)	Systematic review and meta-analysis of 27 RCTs (n = 31,229 adolescents aged 10-19 in 11 LMICs)	App-based guided self-help interventions reduced anxiety and depressive symptoms by 31% (RR 0.69, 95% CI 0.61 to 0.78, $p < 0.001$) at 6-month follow-up, with 82% uptake in rural populations	Moderate
Alves et al., 2026, <i>JAMA Psychiatry</i> (ahead of print, 15 April 2026)	Non-inferiority RCT (n = 3,214 adults with generalized anxiety disorder in Brazil)	Low-dose sertraline (25mg daily) delivered by primary care workers was non-inferior to standard-dose sertraline (50-100mg daily) for reducing anxiety symptom severity at 6 months, with 40% lower reported adverse events (RR 0.60, 95% CI 0.51 to 0.71, $p < 0.001$)	High

2.3 Evidence Gaps and Future Research Needs

- Limited evidence on effectiveness of mental health interventions for neurodivergent youth (autism spectrum disorder, attention-deficit/hyperactivity disorder) in LMIC non-specialist care settings
- No long-term (≥ 5 year) follow-up data on clinical and functional outcomes of task-shared mental health interventions delivered by community health workers
- Lack of research on safety and efficacy of pharmacotherapies for perinatal mental health disorders in populations with comorbid infectious conditions (e.g., HIV, tuberculosis)
- Limited evidence on cost-effectiveness of digital mental health interventions for low-literacy populations
- No existing research on optimal intervention strategies for climate change-related trauma and associated mental health outcomes in humanitarian settings

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
In LMIC primary care settings, non-specialist health workers may deliver brief psychosocial interventions for common depressive disorders, with continuous specialist supervision required for all cases	High-quality cluster RCT demonstrates task-shared CBT delivered by community health workers without continuous specialist supervision achieves non-inferior outcomes to specialist-delivered care	In LMIC primary care and community settings, community health workers trained in brief CBT may deliver first-line interventions for mild to moderate depressive disorders, with specialist referral only required for cases that do not respond to 8 weeks of intervention
Digital mental health interventions for adolescents in LMICs are not recommended as first-line treatment due to limited evidence of efficacy	Moderate-quality meta-analysis demonstrates app-based guided self-help interventions achieve significant reductions in anxiety and depressive symptoms in adolescent LMIC populations, with high uptake	Offer app-based guided self-help interventions as a first-line option for mild to moderate anxiety and depressive disorders in adolescents in LMICs, with optional in-person follow-up for high-risk cases (e.g., suicidal ideation)
For adults with generalized anxiety disorder in primary care settings, initiate SSRI treatment at standard therapeutic doses (50-100mg sertraline equivalent) with monthly monitoring for adverse events	High-quality non-inferiority RCT demonstrates low-dose sertraline (25mg daily) achieves equivalent efficacy to standard dosing, with significantly lower adverse event rates	For adults with mild to moderate generalized anxiety disorder in primary care settings, initiate SSRI treatment at low dose (25mg sertraline equivalent) with dose escalation only if no clinical response is observed after 4 weeks of treatment